

Cannabis as Sacred Medicine

Two-Page Policy / Hearing Testimony Adaptation

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Opening Statement

My name is Michael W. Hughes of Parallax / PHI369 Labs. I submit this statement in support of a cannabis policy framework that recognizes medical value, repairs prohibition harm, protects vulnerable people, and treats cannabis as a powerful plant medicine requiring evidence, humility, and responsible access.

1. Why This Matters

Cannabis policy has too often moved between two extremes: criminalization rooted in stigma and commercialization driven by profit. Both fail patients, families, clinicians, and communities. The better path is a sacred medicine framework: power held with reverence, evidence, consent, maturity, cultural humility, and harm-reduction boundaries.

- Medical value is real but bounded. Evidence is strongest for selected therapeutic contexts, including chronic pain in adults, chemotherapy-related nausea/vomiting, and patient-reported MS spasticity symptoms.
- Research barriers created a Schedule I catch-22: cannabis was labeled as having no accepted medical use while the studies needed to establish accepted use were made unusually difficult.
- Patient testimony matters, especially from people with chronic pain, opioid exposure histories, veterans, palliative-care patients, and those harmed by stigma.
- Rescheduling is not repair. Record clearing, provider education, research access, and stigma reduction must accompany any federal change.

2. Lived-Experience Position

I approach this issue as someone shaped by severe chronic pain, opioid exposure, methadone-clinic stigma, and long-term cannabis use. My testimony is not universal medical advice. It is one lived example of why cannabis users should not be reduced to criminals, stereotypes, or consumers. Many people were using a plant that helped them survive, and many were punished, shamed, or silenced for it.

Policy Thesis

Cannabis is sacred medicine when it helps a person return to breath, dignity, responsibility, joy, love - and when that return becomes care for others. Law should protect that possibility while honestly addressing risks.

3. Recommended Actions

Area	Recommendation
Research	Remove barriers to rigorous whole-plant, cannabinoid, terpene, route-of-administration, and long-term outcome studies.
Education	Require practical ECS and cannabinoid education for clinicians, including drug-interaction and stigma-aware communication training.
Justice	Expand expungement, reentry support, and community reinvestment for people harmed by cannabis criminalization.
Safety	Maintain age protections, contaminant testing, potency labeling, adverse-event tracking, CHS monitoring, and impaired-driving education.
Market	Reject profit-maximized intoxication as the default model; regulate marketing, youth-attractive products, and ultra-high-potency novelty products.
Culture	Protect religious/cultural use, patient testimony, peer stewardship, and non-exploitative community education.

4. Requested Public-Health Metrics

Measure	Why It Matters
Patient disclosure rates	Shows whether patients trust clinicians enough to discuss cannabis honestly.
Provider stigma surveys	Measures whether healthcare culture is becoming safer and more accurate.
ECS curriculum adoption	Tracks whether clinicians are being trained in the biological system patients are using.
Youth-use / CHS / driving data	Protects public health without reverting to propaganda.
Expungement and equity outcomes	Measures whether reform repairs past harm rather than merely creating a new industry.

Closing Statement

A wise cannabis policy should neither demonize the plant nor pretend it is harmless. It should tell the truth, repair the wound, protect the vulnerable, and honor the healed.

Selected Sources

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- [4] Justice Department. Marijuana products and June 2026 hearing notice. 2026. <https://www.justice.gov/opa/pr/justice-department-places-fda-approved-marijuana-products-and-products-containing-marijuana>
- [5] Federal Register. Schedules of Controlled Substances: Rescheduling of Marijuana. 2026. <https://www.federalregister.gov/documents/2026/04/28/2026-08177/schedules-of-controlled-substances-rescheduling-of-marijuana>